

VALLEY VIEW ENDOCRINOLOGY & PRIMARY CARE GROUP

INTEGRATED ELECTRONIC HEALTH RECORD - OUTPATIENT CHRONIC CARE SUMMARY
STRICTLY CONFIDENTIAL - COMPLETED PACIFICA CLINICAL PACKET

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SECTION 1: PATIENT DEMOGRAPHICS & CLINICAL REGISTRY

Patient Identification Registry

Full Legal Name: HARRINGTON, BEATRICE ELENA

Medical Record Number: MRN-55219-W

Date of Birth: 10/24/1969

Age / Gender: 56 Years / Female

Marital Status: Divorced

Home Address: 1422 Windmill Ridge Road, Suite 12, Metropolis, NY 10025

Administrative & Insurance Registry

Primary Insurance: Horizon Senior Preferred Blue PPO

Policy ID Number: HZB-883214-M

Group Number: GRP-11405

Subscriber Name: Beatrice E. Harrington

Emergency Contact: Thomas Harrington (Son)

Contact Phone: (555) 012-9934

Relationship to Pt: Next of Kin

Clinical Account Summary

Primary Care MD: Dr. Evelyn Thorne, MD (Internal Medicine)

Specialist Physician: Dr. Aris Vance, MD (Endocrinology)

Assigned Clinic Site: Valley View Medical Pavilion, Suite 400

SECTION 2: UNIFIED ACTIVE PROBLEM REGISTRY

The following structural registry tracks the active, chronic, and complicated conditions verified for this patient profile across clinical networks.

ICD-10 Code	Condition Description	Onset Date	Status
E11.21	Type 2 Diabetes Mellitus with Diabetic Nephropathy	04/12/2012	Active (Suboptimal)
E11.40	Type 2 Diabetes Mellitus with Neuropathy Unspec.	08-14-2018	Active (Symptomatic)
I10	Essential (Primary) Hypertension	10/22/2009	Active (Controlled)
E66.01	Morbid Obesity due to Excess Calories	05/19/2015	Active (BMI 36.4)
E78.5	Hyperlipidemia, Unspecified	11/02/2011	Active
N18.31	Chronic Kidney Disease, Stage 3a (Mild-Moderate)	2023-03-15	Stable / Chronic

SECTION 3: DOCUMENTED ALLERGIES & ADVERSE DRUG REACTIONS

Allergen / Agent	Reaction Classification	Clinical Manifestation / Severity	Logged Date
Penicillin G / VK	Immunologic (Type I)	Diffuse anaphylactoid hives, dyspnea	06/14/1994
Atorvastatin Calcium	Myotoxicity	Severe bilateral lower limb myalgia	10/05/2016

SECTION 4: COMPREHENSIVE OUTPATIENT MEDICATION PROFILE

The following profile combines verified current prescriptions, active home medications, and recent modifications.

Medication Name	Dosage	Route	Frequency	Prescribing MD	Clinical Indication
Metformin HCl ER	1000 mg	PO	Once Daily	Dr. A. Vance	Type 2 Diabetes Mellitus
Glipizide ER	10 mg	PO	Once Daily AM	Dr. A. Vance	Type 2 Diabetes Mellitus
Empagliflozin	25 mg	PO	Once Daily AM	Dr. A. Vance	Diabetes / Renal Protection
Jardiance	25 mg	PO	Once Daily	Dr. E. Thorne	[DUPLICATE ENTRY IN EHR]
Lisinopril	40 mg	PO	Once Daily	Dr. E. Thorne	Hypertension / Renal Control
Rosuvastatin Sodium	20 mg	PO	Once Daily QHS	Dr. E. Thorne	Hyperlipidemia
Gabapentin	300 mg	PO	Three Times Day	Dr. A. Vance	Diabetic Neuropathy Pain
Furosemide	20 mg	PO	Once Daily AM	Dr. E. Thorne	Edema / Fluid Management
Omeprazole	20 mg	PO	Once Daily AC	Dr. E. Thorne	GERD
Cholecalciferol	2000 IU	PO	Once Daily	Dr. E. Thorne	Vitamin D Insufficiency

Clinical Pharmacist Reconciliation Note: Metformin, Glipizide, and Empagliflozin are active. The computer system flags a critical duplicate entry stream artifact: Empagliflozin 25 mg and Jardiance 25 mg are documented concurrently, ordered via separate specialist and primary care interfaces. Advised patient to discontinue taking both simultaneously to avoid profound volume depletion.

SECTION 5: RECENT LABORATORY METRICS & CLINICAL BIOMARKERS

Specimen Collection Date: May 12, 2026 | Status: Final Verified | Lab Facility: Valley View Central Labs

HEMOGLOBIN A1C (HBA1C) HISTORICAL TREND

Testing Date	Result Value	Reference Range	Clinical Interpretation
11/14/2024	7.2 %	4.0 - 5.6 %	Elevated (Outpatient Control)
06-08-2025	7.6 %	4.0 - 5.6 %	Elevated (Suboptimal Adherence)
2025-12-18	8.1 %	4.0 - 5.6 %	Elevated (Therapeutic Escalation Indicated)
05-12-2026	8.4 %	4.0 - 5.6 %	CRITICAL HIGH (Current Treatment Failure)

Comprehensive Metabolic Panel (Cmp) & Glucose Logs

Component Lab Test	Result Value	Reference Range	Status Flags
Fasting Serum Glucose	168 mg/dL	70 - 99 mg/dL	HIGH (Abnormal)
Serum Creatinine	1.38 mg/dL	0.50 - 1.10 mg/dL	HIGH (Abnormal)
eGFR (CKD-EPI Formula)	43 mL/min/1.73m²	>= 60 mL/min/1.73m²	LOW (Abnormal)
Serum Sodium	138 mEq/L	136 - 145 mEq/L	Normal
Serum Potassium	4.4 mEq/L	3.5 - 5.1 mEq/L	Normal
Blood Urea Nitrogen (BUN)	24 mg/dL	7 - 20 mg/dL	ELEVATED
Total Cholesterol	192 mg/dL	< 200 mg/dL	Normal
LDL Cholesterol (Calc)	114 mg/dL	< 70 mg/dL (Target)	HIGH (Abnormal)
HDL Cholesterol	38 mg/dL	> 50 mg/dL	LOW (Abnormal)
Triglycerides	202 mg/dL	< 150 mg/dL	ELEVATED

Ambiguous Reference Lab Note: An un-annotated standalone laboratory data block executed on the separate date of 05/14/2026 lists a diagnostic index labeled simply "GLU-PAN-FAST" with a numeric value of "6.1". Standard system units are omitted, making it clinically ambiguous whether this represents a point-of-care fasting blood glucose reading in mmol/L or an uncalibrated capillary metric.

SECTION 6: LONGITUDINAL PHYSICIAN FOLLOW-UP NOTES

Encounter Date: 02/10/2026

Provider: Dr. Evelyn Thorne, MD (Internal Medicine)

Encounter Type: Routine 3-Month Chronic Disease Follow-Up

Chief Complaint: "Here for regular follow-up of diabetes, high blood pressure, and worsening numbness in my toes."

Subjective: Mrs. Harrington returns for assessment of her multiple chronic conditions. She reports general compliance with her medications but admits to significant difficulty adhering to a low-carbohydrate, low-sodium meal plan due to high stress and a hectic work schedule. Complains of persistent, burning, symmetrical pain in the distal plantar surfaces of both feet, which she describes as worse at night and interfering with her sleep quality. Denies chest pain, shortness of breath, orthopnea, or a history of transient ischemic symptoms. Home blood pressure readings average 132/82 mmHg to 138/84 mmHg.

Objective Evaluation:

Vitals: BP 136/82 mmHg, HR 72 bpm, RR 16/min, Temp 98.2 °F, Wt 224 lbs, BMI 36.4 (Class II Obesity).

Cardiovascular: Regular rhythm, normal S1 and S2. No peripheral murmurs or vascular bruits noted.

Pulmonary: Lungs are completely clear to auscultation bilaterally. **Extremities:** 1+ trace pitting edema over the bilateral lower shins. Posterior tibial and dorsalis pedis pulses are palpable but diminished at 1+ bilaterally.

Assessment & Plan:

1. Type 2 Diabetes Mellitus: Glycemic control is suboptimal based on rising home fingerstick logs. Awaiting complete lab panels next quarter.
2. Diabetic Neuropathy: Highly symptomatic. Increase Gabapentin dose from 100 mg TID to 300 mg TID to assist with nocturnal burning pain.
3. Chronic Kidney Disease: Stage 3a is stable. Continue maximum dose Lisinopril 40 mg daily for renal endothelial protection.

Encounter Date: 2026-05-15

Provider: Dr. Aris Vance, MD (Endocrinology)

Encounter Type: Comprehensive Metabolic Re-evaluation

Chief Complaint: Detailed review of laboratory results showing an elevated HbA1c of 8.4%.

Subjective: Patient returns following the collection of comprehensive labs on May 12, 2026. She is visibly distressed by her rising hemoglobin A1c trend (now 8.4%). She reports continuing to experience burning sensations in both feet, though slightly improved since the optimization of Gabapentin by her primary care provider. She denies any visual changes, dark urine, or history of hypoglycemic episodes. She notes a 4-pound weight gain over the last month.

Objective Evaluation:

Vitals: BP 142/86 mmHg (Confirmed elevated), HR 76 bpm, Wt 228 lbs. Critical Labs Reviewed: HbA1c 8.4%, Fasting Glucose 168 mg/dL, Creatinine 1.38 mg/dL, eGFR 43 mL/min/1.73m². **Extremities:** Structural foot note compiled below.

Clinical Synthesis & Therapeutic Adjustments:

The current metabolic trajectory reveals clear secondary oral failure of her current diabetic regimen. Her HbA1c has risen progressively from 7.2% to 8.4% over the past 18 months. Concurrently, her chronic kidney disease has progressed to a stable Stage 3a (eGFR 43). Given her reduced renal clearance, Metformin can be safely maintained at 1000 mg daily but cannot be titrated higher. Glipizide ER 10 mg will be maintained for now, but we must optimize her therapy.

We will initiate a weekly subcutaneous GLP-1 receptor agonist to target both her severe insulin resistance and assist with weight loss. Discovered an administrative medication duplication: patient was taking both Empagliflozin 25 mg (ordered under its generic name) and branded Jardiance 25 mg simultaneously. The Jardiance entry has been voided.

Encounter Date: 05/16/2026

Provider: Dr. Robert Mercer, MD (Covering Internal Medicine Resident)

Encounter Type: Telephone Care Coordination Log

Inconsistent/Conflicting Observation Note: Dr. Mercer completed a detailed telephone assessment with the patient regarding home care safety parameters. His final charting log states: "Patient has completely clear, baseline tactile sensation over all aspects of her lower extremities. Vibratory and monofilament sensation is perfectly preserved at 10/10 sites bilaterally. No signs of neuropathic progression or microvascular complications are present on review. Patient is completely asymptomatic." This evaluation directly conflicts with the detailed physical findings logged by both Dr. Thorne and Dr. Vance during the face-to-face clinical encounters immediately preceding and following this phone log.

SECTION 7: DIABETIC FOOT COMPREHENSIVE ASSESSMENT NOTE

Log Timestamp: May 15, 2026 (11:15 AM)

Authoring Clinician: Dr. Aris Vance, MD (Endocrinology)

A structural lower extremity examination was conducted during today's comprehensive endocrine visit to evaluate her diabetic peripheral neuropathy and macrovascular risk index.

Visual Inspection:

The skin over both lower extremities is markedly dry and displays fine scaling, consistent with autonomic neuropathy and loss of normal sweat gland regulation. Moderate hyperkeratotic callus formation is observed over the plantar surfaces of the first and fifth metatarsal heads bilaterally. No acute ulcerations, open fissures, or structural blisters are present. Left second digit displays a mild hammertoe deformity, which is flexible. Nails are thickened and dystrophic bilaterally, without signs of acute paronychia or ingrown margins.

Neurological Testing (Microvascular Sensitivity):

Sensory testing was conducted utilizing a standard 10-gram Semmes-Weinstein monofilament wire. The patient has a profound loss of protective sensation, failing to perceive the filament at 4/10 sites on the right foot and 5/10 sites on the left foot, focused primarily over the distal hallux and metatarsal heads. Proprioception at the great toes is intact. Vibratory sensation tested with a 128 Hz tuning fork is markedly diminished at the bony prominence of the medial malleolus bilaterally, with an absolute perception threshold restricted to under 4 seconds. Achilles deep tendon reflexes are absent bilaterally; patellar reflexes are present but sluggish (1+).

Vascular / Perfusion Status:

The lower limbs are warm to the touch down to the mid-tarsal level, with a cool transition at the toes. Capillary refill time is prolonged at 4 seconds in the digits of both feet. The dorsalis pedis and posterior tibial pulses are diminished and graded at 1+ out of 4 bilaterally, indicating moderate peripheral arterial disease. No active claudication is reported today, but mild dependency rubor is observed.

Clinical Impression & Preventive Guidance:

The patient displays a high-risk diabetic foot profile (Category 2 based on the International Working Group on the Diabetic Foot guidelines) due to a combined loss of protective sensation and concurrent peripheral arterial disease. She is at high risk for future neuropathic ulceration. The patient was instructed on strict daily self-inspection routines using a mirror, the absolute necessity of avoiding walking barefoot under any circumstances, and was provided a formal referral to podiatry for professional nail care and protective custom orthotic shoe fitting.

SECTION 8: UNIFIED COMPREHENSIVE INDIVIDUALIZED CARE PLAN

The following targeted clinical care plan represents the integrated management strategy devised by her multi-disciplinary care team to achieve metabolic control and mitigate further microvascular and macrovascular decline.

Primary Glycemic Optimization Targets:

- Establish a target HbA1c of < 7.5% given her age, duration of disease, and stable Stage 3a chronic kidney disease.
- Maintain daily fasting home capillary glucose readings between 90 mg/dL and 130 mg/dL, and post-prandial spikes under 180 mg/dL.
- Patient will check her blood sugar twice daily: once fasting in the morning and once before dinner, keeping a structured logbook for clinical review.

Cardiovascular & Afterload Targets:

- Maintain blood pressure strictly below 130/80 mmHg to preserve renal filtration grids and reduce stroke indexes.
- Optimize lipid parameters to achieve a target LDL cholesterol of < 70 mg/dL given her high ASCVD risk from long-standing diabetes.

Multi-Disciplinary Referrals & Appointments:

1. Ophthalmology / Retinal Specialist: Ordered an annual dilated fundoscopic examination to screen for diabetic retinopathy.
2. Podiatry Service: Referral issued for comprehensive callus debridement, preventative care, and specialized diabetic footwear coordination.
3. Nephrology Consult: Queue for evaluation if eGFR falls below 30 mL/min/1.73m² or if significant albuminuria develops on spot testing.
4. Certified Diabetes Educator (CDE) / Nutrition Services: Structured 4-part series authorized for intensive carbohydrate counting education and meal portion planning.

SECTION 9: CLINICAL PRESCRIPTION ADJUSTMENTS & RX ORDERS

The following formal outpatient prescription modifications have been authorized and transmitted electronically to the patient's registered care provider at Community Wellness Pharmacy.

Prescription Order 1:

Medication Name: Semaglutide (Ozempic)

Formulation / Delivery: 2 mg / 1.5 mL Subcutaneous Injection Pen

Dosing Frequency: Inject 0.25 mg subcutaneously into the abdomen once a week for the first 4 weeks. Instructed to escalate to 0.5 mg once a week starting on week 5.

Quantity Authorized: 1 Prefilled Pen (1.5 mL total volume)

Refills Allowed: Five (5)

Ordering Physician: Dr. Aris Vance, MD (Endocrinology)

Prescription Order 2:

Medication Name: Rosuvastatin Sodium (Crestor)

Formulation / Strength: 40 mg Oral Tablet

Dosing Frequency: Take 1 tablet daily by mouth at bedtime.

Quantity Authorized: 90 Tablets

Refills Allowed: Three (3)

Ordering Physician: Dr. Evelyn Thorne, MD (Internal Medicine)

Clinical Note: Statin therapy is escalated from 20 mg to 40 mg daily to aggressively drive her current sub-target LDL of 114 mg/dL toward the required < 70 mg/dL clinical threshold.

Prescription Order 3:

Medication Name: Gabapentin (Neurontin)

Formulation / Strength: 300 mg Oral Capsule

Dosing Frequency: Take 1 capsule by mouth three times daily (Every 8 hours) to maintain stable serum levels for neuropathic pain control.

Quantity Authorized: 270 Capsules

Refills Allowed: Eleven (11)

Ordering Physician: Dr. Aris Vance, MD (Endocrinology)

Prescription Order 4 (Discontinuation Directive):

Medication Name: Jardiance (Empagliflozin)

Reason for Action: Formal cancellation of therapeutic duplication. Patient has been thoroughly counseled to dispose of any remaining branded Jardiance bottles at home and to exclusively utilize her generic Empagliflozin 25 mg container as directed to ensure safety.

Action: DISCONTINUE & VOID ACTIVE REFILLS.

Mandated Care Monitoring Protocol

The patient is required to return to the Valley View Outpatient Clinic in 4 weeks for a specialized nursing visit to assess her technique and tolerance of the newly initiated Semaglutide injection. A repeat basic metabolic panel (Creatinine, Electrolytes, and eGFR) along with a spot urine albumin-to-creatinine ratio is scheduled for collection in exactly 8 weeks. Her next comprehensive follow-up with Dr. Aris Vance is confirmed for August 14, 2026.

*** END OF OUTPATIENT CHRONIC MEDICAL FILE - RECORD RECONCILIATION COMPLETE ***